

Dabrafenib and trametinib

Dabrafenib (Tafinlar®) and trametinib (Mekinist®) are cancer drugs. They are used together to treat melanoma and non-small cell lung cancer (NSCLC).

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What are dabrafenib and trametinib?

Dabrafenib and trametinib are used together to treat [melanoma](#) and [non-small cell lung cancer \(NSCLC\)](#). When used for melanoma, this treatment can be given:

- after surgery for [stage 3](#) melanoma to help prevent the cancer coming back
- when melanoma cannot be removed with surgery
- when melanoma has spread to other organs in the body

Dabrafenib and trametinib are types of targeted therapy known as cancer growth inhibitors.

Dabrafenib and trametinib are used if cancer cells have a change (mutation) in a gene called BRAF. The BRAF gene causes the cells to produce a protein that encourages the cancer to grow. Dabrafenib and trametinib block this protein.

It is best to read this information with our general information about [targeted therapy](#) drugs and the [type of cancer](#) you have.

Your cancer team will talk to you about this treatment and its possible side effects before you agree ([consent](#)) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the [electronic Medicines Compendium \(eMC\)](#) website, which has patient information leaflets (PIL) for individual drugs.

How dabrafenib and trametinib are given

Dabrafenib is taken as capsules and trametinib is taken as tablets. The nurse or pharmacist will give you the capsules and tablets to take at home.

During treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

During treatment, you will have regular blood tests. This is to check that it is safe for you to have treatment.

You will speak with a doctor, nurse or pharmacist regularly during treatment. They will talk to you about your blood results and ask how you have been feeling. If your blood results are okay, the pharmacy team will prepare your targeted therapy.

Your course of treatment

How long you take this treatment for will depend on your situation. If you are taking dabrafenib and trametinib after surgery to reduce the risk of melanoma coming back, you have this treatment for up to 1 year.

If the cancer cannot be removed with surgery or is advanced, you take this treatment for as long as it is working for you and any side effects can be managed. Do not stop taking your treatment unless your doctor tells you to.

Your doctor, nurse or pharmacist will discuss your treatment plan with you.

Taking dabrafenib and trametinib

The nurse or pharmacist will give you dabrafenib capsules and trametinib tablets to take home. They may give you capsules and tablets of different strengths. Always take them exactly as they tell you to. This is important to make sure they work as well as possible for you.

Make sure you:

- do not eat anything for 2 hours before taking the capsules and tablets and for 1 hour after
- swallow them whole with a glass of water
- do not chew, open, break or crush them
- take them at the same times every day.

While taking this treatment, you should not have grapefruit or Seville oranges.

This means eating them, drinking the juice or taking any supplements that might contain them. This is because it can increase the amount of dabrafenib in your body and may cause more side effects.

Dabrafenib

You usually take dabrafenib capsules twice a day, 12 hours apart.

If you forget to take dabrafenib and there are more than 6 hours until your next dose, take it as soon as you remember.

If there are less than 6 hours until your next dose, do not take it. Just take the next dose at the usual time. Do not take a double dose to make up for any missed ones.

Trametinib

You usually take trametinib once a day at the same time as either the morning or the evening dose of dabrafenib. If you forget to take trametinib and there are more than 12 hours until your next dose, take it as soon as you remember. If there are less than 12 hours until the next dose, do not take it. Just take the next dose at the usual time. Do not take a double dose to make up for any missed ones.

Never take a double dose of either drug. If you are sick after taking either drug, do not take another dose. Just take the next dose at the usual time. You can contact the hospital on the 24-hour number you have been given for advice.

Other things to remember about your capsules and tablets:

- Keep dabrafenib capsules in the original package with the lid tightly closed. Store at room temperature, away from heat, moisture and direct sunlight. There is a small cylinder container in the bottle to help keep the capsules dry. This must stay in the bottle with the capsules.
- Keep trametinib tablets in the original package with the lid tightly closed. Store at room temperature (not higher than 30°C) away from heat, moisture and direct sunlight. Once a new bottle is opened, you can keep it for up to 30 days. There is a small cylinder container in the bottle to help keep the tablets dry. This must stay in the bottle with the tablets.
- Keep them safe, where children cannot see or reach them.
- Do not throw away unused capsules and tablets. Return them to your cancer team at the hospital.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Always tell your doctor, nurse or pharmacist about any side effects you have. They can give you:

- drugs to help control some side effects
- advice about managing side effects.

It is important to take any drugs exactly as explained. This means they will be more likely to work for you.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Very common side effects

These side effects happen to 10 or more people in every 100 people (10% or more) who have this treatment.

High temperatures (fever)

This treatment can cause high temperatures. This usually starts in the first month of treatment. A high temperature can cause symptoms such as:

- night sweats
- feeling hot or cold
- feeling shivery
- feeling achy.

Contact the hospital straight away if you have these symptoms. You will usually be told to check your temperature. A high temperature can also be caused by infection, so your doctor or nurse may ask you to come to the hospital for a check-up.

If the treatment is causing a high temperature, your doctor may tell you to stop taking dabrafenib for a while, or to take a lower dose of it.

Swelling of nasal passages and back of throat

When taking this treatment, your nose may feel stuffy or blocked, and you may have a sore throat.

Feeling tired

[Feeling tired](#) is a common side effect of this treatment. It is often worse towards the end of treatment and for some weeks after it ends. Try to pace yourself and plan your day so you have time to rest. Gentle exercise, like short walks, can help you feel less tired.

If you feel sleepy, do not drive or use machinery.

Feeling sick

You may feel sick during this treatment, but this is usually mild. Your doctor, nurse or pharmacist may prescribe you anti-sickness drugs to help prevent or control sickness. Take the drugs exactly as they tell you to. [If you feel sick](#), take small sips of fluid often and eat small amounts regularly. It is important to drink enough fluids.

If you continue to feel sick, or if you are sick (vomit) 1 to 2 times in 24 hours, contact the hospital on the 24-hour number as soon as possible. They will give you advice.

Bleeding

This treatment can cause bleeding, which may be mild or more serious.

It can also reduce the number of platelets in your blood. Platelets are cells that help the blood to clot. If the number of platelets is low, you may bruise or bleed easily. You may need a drip to give you extra platelets. This is called a platelet transfusion.

Contact the hospital straight away on the 24-hour number if you have any of these symptoms:

- severe headaches or dizziness
- blood in your stools (poo) or black 'tarry' stools
- vomit that looks dark and grainy like coffee grounds
- sudden tummy pain
- coughing up blood
- blood in the urine (pee)
- nose bleeds
- bleeding gums
- heavy periods
- tiny red, brown or purple spots that may look like a rash – these spots can be harder to see if you have black or brown skin.

Diarrhoea

This treatment may cause [diarrhoea](#). Diarrhoea means passing more stools (poo) than is normal for you, or having watery or loose stools. You may also have stomach cramps. If you have a stoma, it may be more active than usual.

If you are passing loose stools 3 or more times a day and this is not normal for you, contact the hospital as soon as possible on the 24-hour number. Follow the advice they give you about:

- taking anti-diarrhoea medicines
- drinking enough fluids to keep you hydrated and to replace lost salts and minerals
- any changes to your diet that might help.

They might also ask you for a specimen of your stool to check for infection.

Constipation

This treatment can cause [constipation](#). Constipation means that you are not able to pass stools (poo) as often as you normally do. It can become difficult or painful. Here are some tips that may help:

- Drink at least 2 litres (3½ pints) of fluids each day.
- [Eat high-fibre foods](#), such as fruit, vegetables and wholemeal bread.
- Do regular gentle exercise, like going for short walks.

If you have constipation, contact the hospital on the 24-hour number for advice. They can give you drugs called laxatives to help.

If you have not been able to pass stools for over 2 days and are being sick, contact the 24-hour number straight away.

Tummy pain

You may get pain in your tummy (abdomen). Your doctor, nurse or pharmacist can give you advice and suggest treatments to help. Tell them if the pain gets worse or does not get better.

Rarely tummy pain can be a sign of a more serious problem linked to the pancreas or to a hole in the bowel lining. If you develop severe tummy pain, contact the hospital for advice straight away.

Loss of appetite

This treatment can affect your appetite. Don't worry if you do not eat much for 1 or 2 days. But if your appetite does not come back after a few days, or if you are losing weight, tell your doctor, nurse or pharmacist. They can give you advice. They may give you food or drink supplements. Or they may suggest changes to your diet or eating habits to help.

Headache and dizziness

You may have headaches or feel dizzy. If this happens, let your doctor or nurse know. They can do tests to check for the cause. If a headache or dizziness is severe, contact the hospital for advice straight away.

If you feel dizzy, do not drive or use machinery.

Muscle or joint pain

You may have muscle or joint pain, or muscle spasms (cramps). The pain is usually mild and can be relieved with painkillers. Ask your doctor for advice.

Rarely, this treatment can cause a condition where muscle breaks down and affects the kidneys. Always tell your doctor if:

- muscle pain does not get better
- muscle pain is causing weakness
- your urine (pee) is darker than normal.

Effects on the lungs

This treatment can cause inflammation of the lungs. This is called pneumonitis. Contact the hospital straight away on the 24-hour number if you notice any of these changes during treatment or after it ends:

- breathlessness
- a cough that does not go away
- wheezing
- a fever, with a temperature over 37.5°C.

You should also tell them if any existing breathing problems get worse. You may have:

- tests to check your lungs
- treatment, if needed.

Changes in your blood pressure

Dabrafenib and trametinib can affect your blood pressure. It may make cause high or sometimes low blood pressure. Your blood pressure will be checked before and during treatment. High blood pressure can usually be controlled with tablets.

Skin changes

This treatment often causes mild skin changes. They include:

- a rash
- redness – if you have black or brown skin this can be harder to notice
- itching
- dry skin

Less commonly, this treatment can cause

- rough or scaly skin
- small wart-like growths (called papillomas)
- infections in the skin or nails.

Tell your nurse or doctor if you have any of these changes. They can give you advice, and can prescribe creams and medicines to help.

If you are out in the sun, use a sun cream with a high sun protection factor of at least SPF 30 to protect your skin.

Fluid build-up

Some people notice their hands and feet become swollen. This is caused by a build-up of fluid. If you notice this, tell your doctor or nurse so they can give you advice and treatment to help.

Less commonly, this treatment can cause redness or swelling around the eyes. Contact the hospital straight away if you notice this.

Effects on the liver

This treatment may affect how your liver works. This is usually mild and goes back to normal after treatment ends. You will have blood tests to check how your liver is working.

Tumour lysis syndrome (TLS)

Some people are at risk of developing a condition called tumour lysis syndrome (TLS). The risk is highest at the beginning of treatment. TLS happens when treatment makes large numbers of cancer cells die and break down quickly. This releases lots of waste products into the blood and can affect the kidneys and heart.

TLS can be prevented or treated. You will have regular blood tests to check for TLS. You may have:

- extra fluids through a drip
- medicines such as allopurinol tablets or rasburicase through a drip.

Drinking at least 2 litres (3½ pints) of fluid a day will also help.

Other side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Risk of infection

This treatment can reduce the number of white blood cells in your blood. These cells fight infection. If your white blood cell count is low, you may be more likely to get an [infection](#). A low white blood cell count is called neutropenia.

An infection can be very serious when the number of white blood cells is low. It is important to get any infection treated as soon as possible. If you have any of the following symptoms, contact the hospital straight away on the 24-hour number:

- a temperature above 37.5°C
- a temperature below 36°C
- you feel unwell, even with a normal temperature
- you have symptoms of an infection.

Symptoms of an infection include:

- feeling shivery and shaking
- a sore throat
- a cough
- breathlessness
- diarrhoea
- needing to pass urine (pee) often, or discomfort when you pass urine.

It is important to follow any specific advice your cancer treatment team gives you.

You will have regular blood tests during treatment. If your white blood cell count is low, your doctor may stop your treatment for a short time, until your cell count increases. Sometimes they will reduce the dose of your treatment.

Anaemia (low number of red blood cells)

This treatment can reduce the number of red blood cells in your blood. Red blood cells carry oxygen around the body. If the number of red blood cells is low, this is called anaemia. You may feel:

- very low in energy
- breathless
- dizzy and light-headed.

If you have these symptoms, contact the hospital straight away on the 24-hour number. You may need treatment for anaemia. If you are very anaemic, you may need a drip to give you extra red blood cells. This is called a blood transfusion.

Hair loss

You might notice that your hair slowly gets thinner. But you are very unlikely to lose all the hair from your head. Your hair can also become more brittle. Your nurse can give you information about [coping with hair loss](#). Your hair will usually go back to normal after treatment ends.

Hand-foot (palmar-plantar) syndrome

This treatment can affect the palms of your hands and the soles of your feet. This is called palmar-plantar or hand-foot syndrome.

If you have white skin these areas may become red. If you have black or brown skin, these areas might get darker.

The skin on the palms of your hands and the soles of your feet may:

- be sore
- be painful, tingle, or swell
- peel, crack or blister.

If you have any of these symptoms, contact the hospital straight away on the 24-hour number. They can give you advice. This is especially important if you have any broken skin or if walking is difficult. They can prescribe creams and painkillers to help.

You can care for your hands and feet by:

- keeping your hands and feet cool by washing in cool water
- gently moisturising your hands and feet regularly
- wearing gloves to protect your hands and nails when working in the house or garden
- wearing loose cotton socks and avoiding tight-fitting shoes and gloves.

Serious skin changes

Rarely, this treatment can cause a serious skin reaction that needs to be treated immediately in hospital. Contact the hospital straight away on the 24-hour number if you have any of these symptoms:

- a skin rash that is spreading
- blistering or peeling skin
- flu-like symptoms, such as a high temperature and joint pain
- sores on your lips or in your mouth.

Dry or sore mouth

Your mouth may become dry or sore. Tell your nurse or doctor if you have any problems with your mouth. They can prescribe medicines to prevent or treat mouth infections and reduce any soreness. It is important to follow any advice you are given and to drink plenty of fluids.

New skin cancer

Dabrafenib and trametinib increase the risk of [skin cancers](#). Your doctor will check your skin before and during treatment.

If a skin cancer develops it can usually be removed with surgery. Your nurse or doctor will tell you what to look for and will check your skin regularly. If you notice anything unusual between appointments, let your nurse or doctor know.

Eye problems

This treatment may affect the eyes. Your doctor will check your eyes while you are taking this treatment.

Contact the hospital straight away if you notice:

- any changes in your vision, such as blurring or loss of vision
- any eye pain or redness
- that your eyes have become more sensitive to light.

If your eyes are affected, you may need to have the dose of trametinib reduced or stopped. It is important that any eye problems are treated quickly. Your doctor may refer you to an eye specialist. Do not drive if your vision is affected.

Effects on the kidneys

This treatment can affect how the kidneys work. This is usually mild and goes back to normal after treatment ends. You will have blood tests to check how well your kidneys are working. Contact the hospital on the 24-hour number if you:

- have blood in your urine (pee)
- are passing less urine or peeing less often than usual.

Drinking fluids helps protect your kidneys. The advice is usually to try to drink at least 2 litres (3½ pints) of fluid each day. But follow any advice from your doctor, nurse or pharmacist about how much is right for you.

Raised blood sugar levels

This treatment can raise your blood sugar levels and increase the risk of diabetes.

Signs of raised blood sugar include:

- feeling thirsty
- needing to pass urine (pee) more often than usual
- feeling more tired than usual.

Tell your doctor or nurse if you have these symptoms. They can do tests to check your blood sugar level and give you advice.

If you already have [diabetes](#), your blood sugars may be higher than usual. You may need to check them more often, or make changes to your diabetic treatment. Your diabetic doctor or nurse will talk to you about how to manage this.

Numb or tingling hands or feet (peripheral neuropathy)

This treatment may affect the nerves in your fingers and toes. This can cause numbness, tingling or pain in your hands or feet. This is called [peripheral neuropathy](#). You might find it hard to do fiddly tasks such as fastening buttons or tying shoelaces.

If you have these symptoms, always tell your doctor, nurse or pharmacist. They sometimes need to change the drug or the dose of the drug. The symptoms usually improve slowly after treatment ends. But for some people they continue and are a long-term side effect of treatment.

Effects on the heart

This treatment can affect how the heart works. You may have [tests](#) to check how well your heart is working. These may be done before, during and after treatment.

If the treatment is causing [heart problems](#), your doctor may change the type of treatment you are having.

Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- breathlessness
- dizziness
- changes to your heartbeat (palpitations)
- swollen feet and ankles.

Other conditions can cause these symptoms, but it is important to get them checked by a doctor.

Always call **999** if you have:

- chest pain, pressure, heaviness, tightness or squeezing across the chest
- difficulty breathing.

Other important information

Blood clot risk

Cancer and some cancer treatments can increase the risk of a [blood clot](#). Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
-

reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker

- suddenly feeling breathless or coughing.

Always call **999** if you have:

- chest pain
- difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- [herbal drugs and complementary or homeopathic therapies](#)
- recreational drugs – for example, cannabis.

Vaccinations

Cancer doctors usually recommend that people with cancer have vaccinations for flu and [coronavirus \(covid\)](#). They may also recommend other vaccines, such as Shingrix[®] for shingles. These all help reduce your risk of serious illness from these infections. Most people can have these vaccines, including people with weak immune systems.

You should not have live vaccines if your immune system is weak. This includes if you are having or recently had chemotherapy, radiotherapy or other cancer treatments that affect your immune system. Live vaccines can make you unwell because they contain a very weak version of the illness they protect you against. There are several live vaccines, including the yellow fever vaccine.

It is important to ask your doctor, nurse or pharmacist for advice about having vaccinations. They can explain what vaccines are right for you and when it is best to have them.

Contraception

Your doctor, nurse or pharmacist will advise you not to [get pregnant](#) or [make someone pregnant](#) while having this treatment and for some time afterwards. The drugs may harm a developing baby. It is important to use contraception to prevent pregnancy. Follow their advice about:

- what types of contraception to use
- how long after treatment you should continue to use contraception.

Breastfeeding

You are advised not to [breastfeed](#) while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

Fertility

Some cancer drugs can affect whether you can [get pregnant](#) or [make someone pregnant](#). If you are worried about this, it is important to talk with your doctor before you start treatment.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Visit the [electronic Medicines Compendium \(eMC\)](#) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

[Cancer Information Development team](#)

Author

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help